



Nursing Process

By: Fantahun Guteta (BscN, Mph)



Nursing Process



- One definition of the nursing process...”**an assertive, problem solving approach** to the identification and treatment of patient problems.
- A process is a series of steps or acts that lead to accomplishment of some goal or purpose.

Purposes of nursing process



1. Providing professional, **quality nursing** care.
2. Directs nursing activities for health promotion, health protection, and disease prevention and is used by nurses in every practice setting and specialty.
3. Provides the basis for critical thinking in nursing.

Purpose of the nursing process...Cont'd

4. It is a basis for problem solving, clinical decisions, and individualized patient care

4. It uses the scientific method of:
 - ✓ Observation,
 - ✓ Measurement,
 - ✓ Data collection, and
 - ✓ Data analysis to evaluate the needs of patients and their families

Components of Nursing Process

- The nursing process consists of **five dynamic and interrelated phases**.

1. *Assessment*
2. *Diagnosis*
3. *Planning*
4. *Implementation*
5. *Evaluation*



Assessment



1. Assessment



- **Nursing Assessment:** the first phase/initial step of nursing process that put the baseline data for the rest of nursing process.
- It is the systematic and continuous
 - Systematic data Collection,
 - Validation,
 - Interpretation (analysis)
 - Organization and
 - Documentation of data about a patient/client.

Systemic collection of data



❑ Sources of Data collection by assessment methods:

1. Primary sources
2. Secondary sources:



Types of data collection

1. Subjective data/symptom
2. Objective data /sign



Nursing Assessment...Cont'd

- The data/information about the patient/client will be collected using the skills of:
 - ☐ Observation,
 - ☐ Interviewing,
 - ☐ Physical examination.



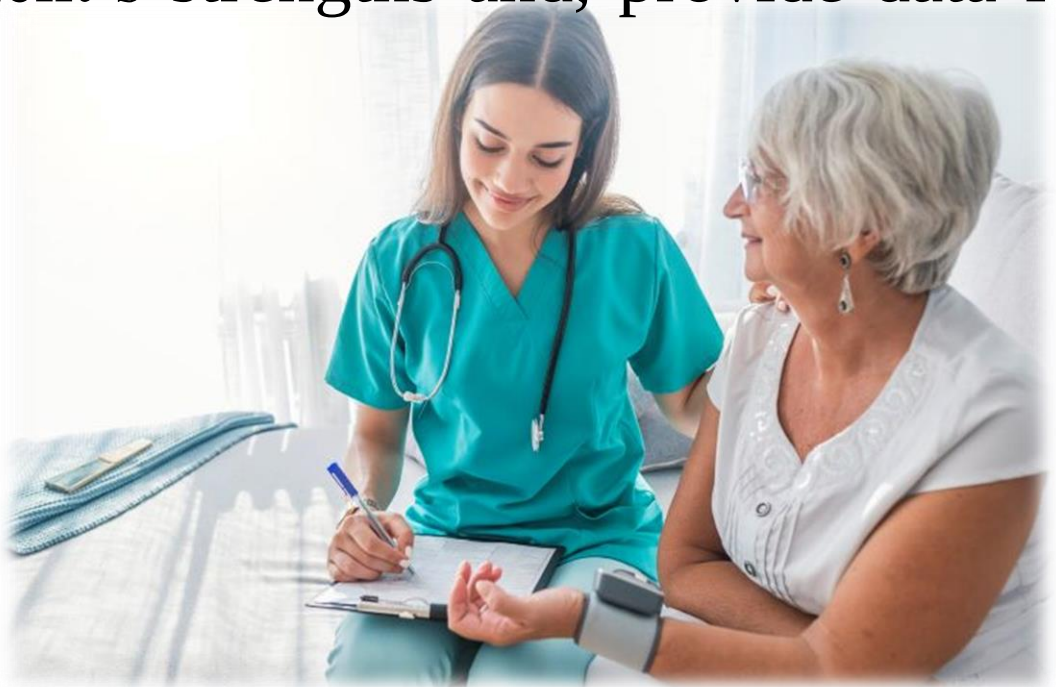
Types of Assessment

1. Initial assessment
2. Focus assessment
3. Emergency assessment
4. Time – lapsed reassessment



Reason/purpose of nursing assessment

- ❑ To establish baseline information on the client
- ❑ To determine the client's normal function and, risk for dysfunction
- ❑ To determine the client's strengths and, provide data for the diagnosis phase



Physical examination

- **Physical examination:** is the process of evaluating objective anatomic findings through the use of
 - Observation/inspection,
 - Palpation,
 - Percussion, and
 - Auscultation.



1. Inspection



- Is the critical observation and most frequently used assessment technique using **sense organs to inspect skin color, location of lesions, odor etc.**
- Close and careful visualization of the person as a whole and of each body system.



2. Palpation

- It is the technique of touching the patient using different parts of hands and exerts pressure in different strength.
 1. **Light palpation.**
 2. **Deep palpation**



2. Palpation...Cont'd

- **Light palpation** allows you to assess for texture, tenderness, temperature, moisture, pulsations, and masses.
- **Deep palpation** is performed to assess for masses and internal organs.



Light palpation



Deep palpation

3. Percussion

- Is used to elicit tenderness or sounds that may provide clues to underlying problems. It requires skill and practice, monitor sign of discomfort.
- It assesses the underlying structures for **location, size, density of underlying tissue.**



The common percussion sounds produced

- **Resonance:** A hollow sound.
- **Hyper resonance:** A booming sound.
- **Tympany:** A musical sound or drum sound like that produced by the stomach.
- **Dullness:** Thud sound produced by dense structures such as the liver, and enlarged spleen, or a full bladder.
- **Flatness:** An extremely dull sound like that produced by very dense structures such as muscle or bone.

The common percussion sounds...Cont'd

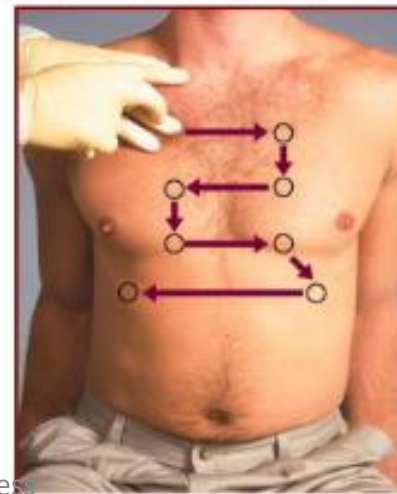
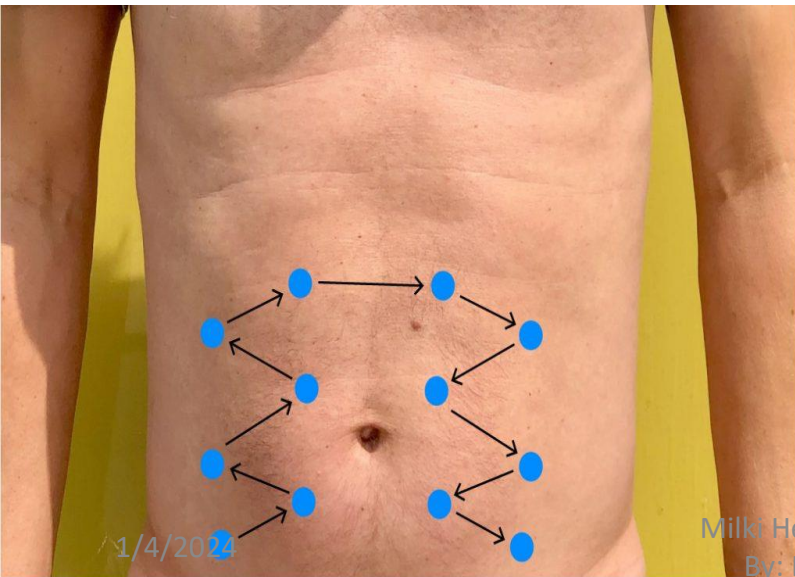
- **Resonance (heard over partial)**= Normal lung
- **Hyper-resonance (heard over mostly air)**=Lung with emphysema
- **Tympany (heard over air)**=Puffed-out cheek, gastric bubble
- **Dullness (heard over more solid tissue)**=Diaphragm, pleural effusion
- **Flatness** (heard over very dense tissue) =Muscle, Bone, Thigh

The areas where percussion should be performed

- Chest
- Abdomen
- Back
- Head



Percuss the anterior chest.



4. Auscultation



4. Auscultation

- **Auscultation:** Listening to sounds produced by the body using an instrument called stethoscope.
- It is usually performed following inspection especially **with abdominal assessment.**



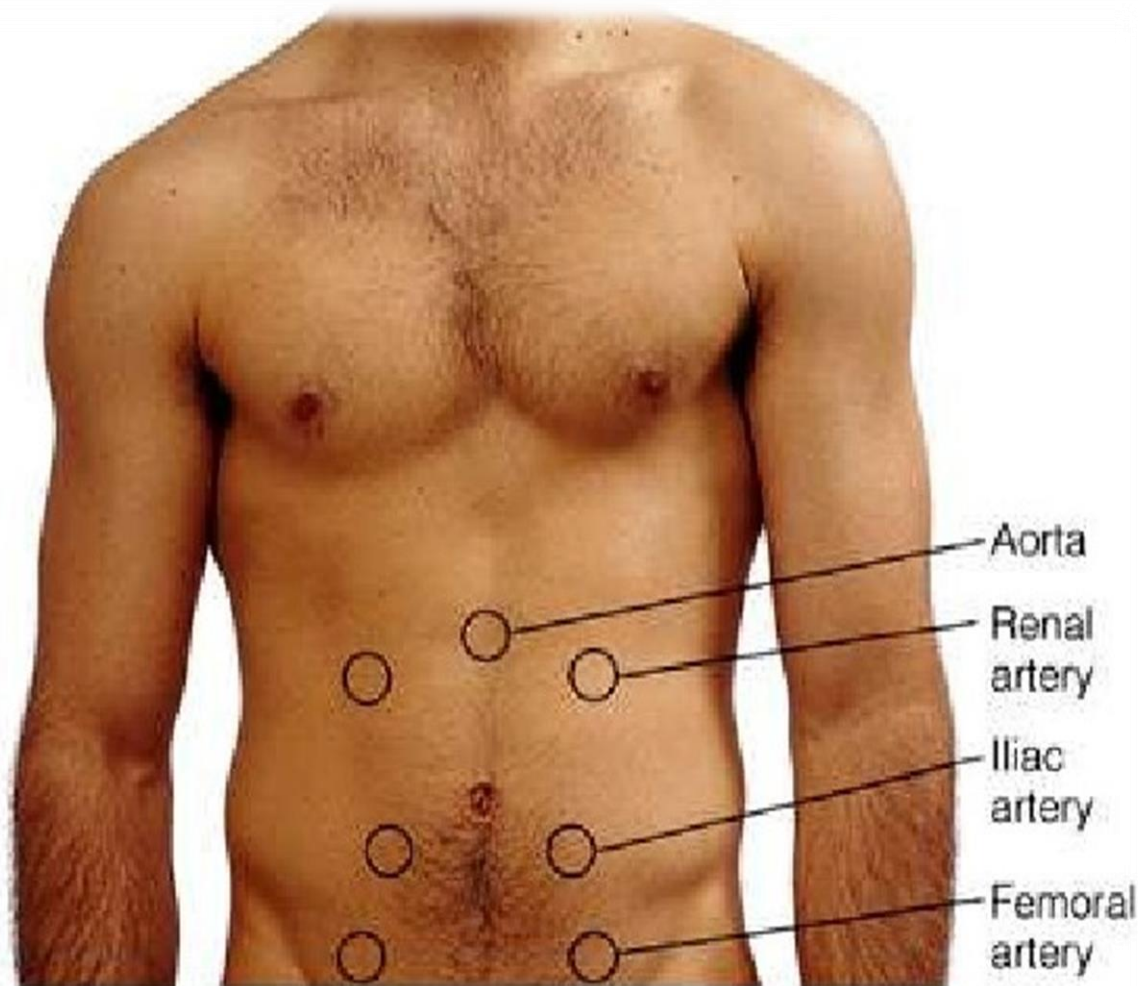
The areas where auscultation should performed

- ☐ Abdomen
- ☐ Thyroid Gland
- ☐ Heart
- ☐ Lung

Site for auscultation the Thyroid Gland

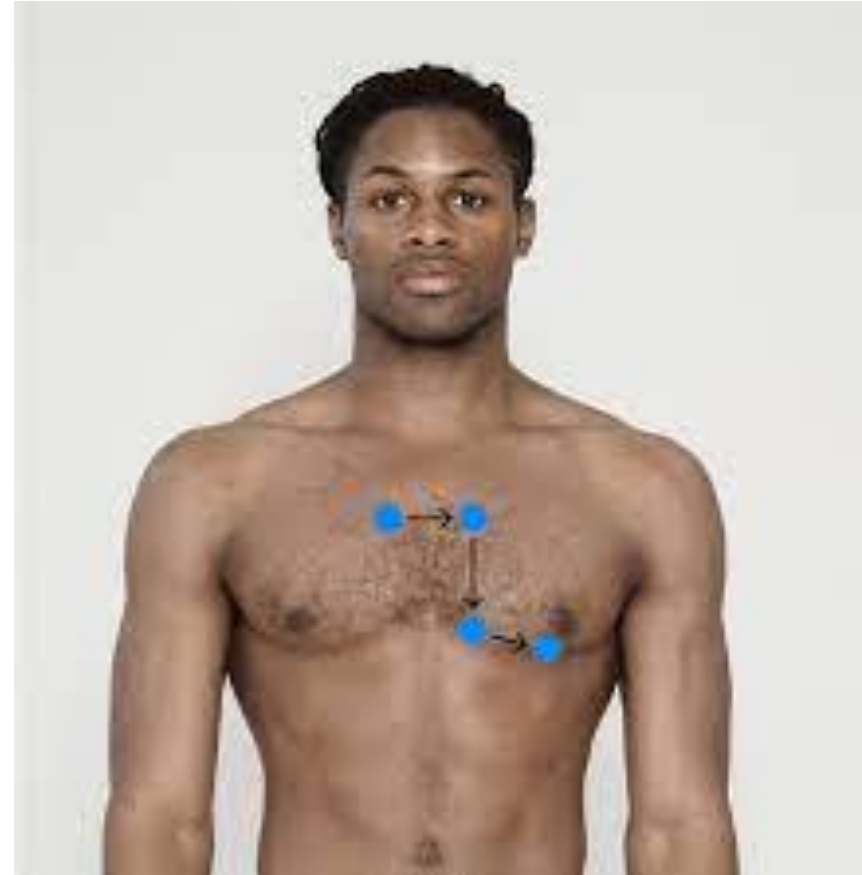
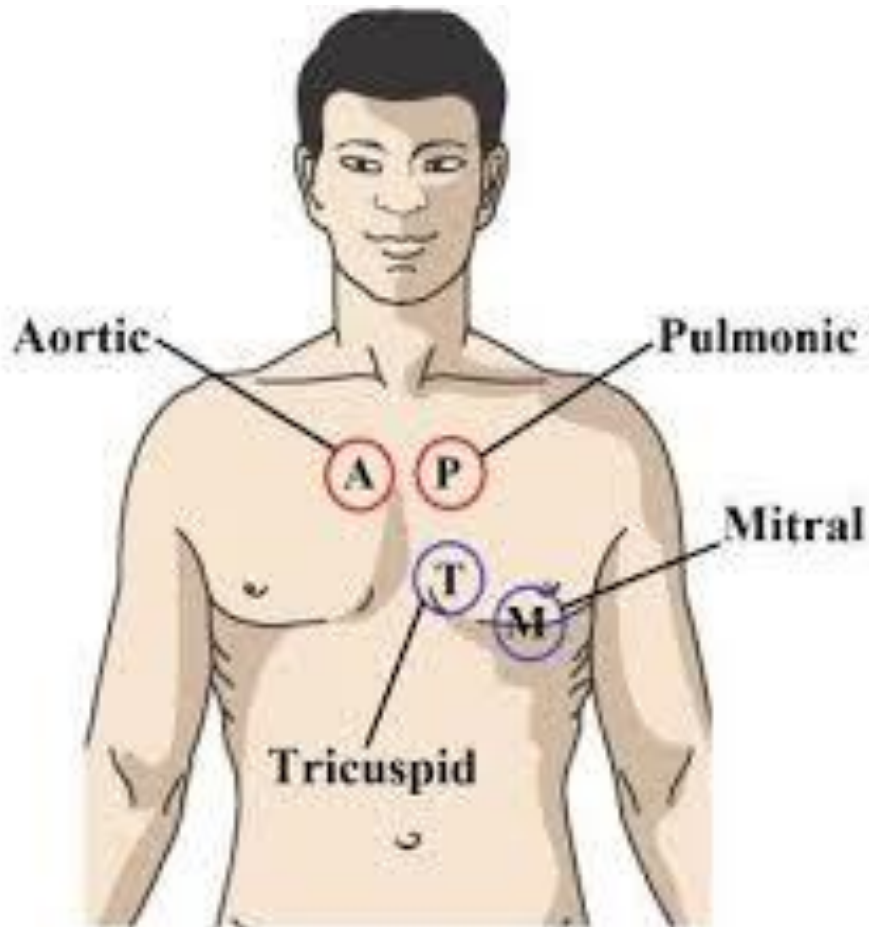


Site for auscultation the Abdomen

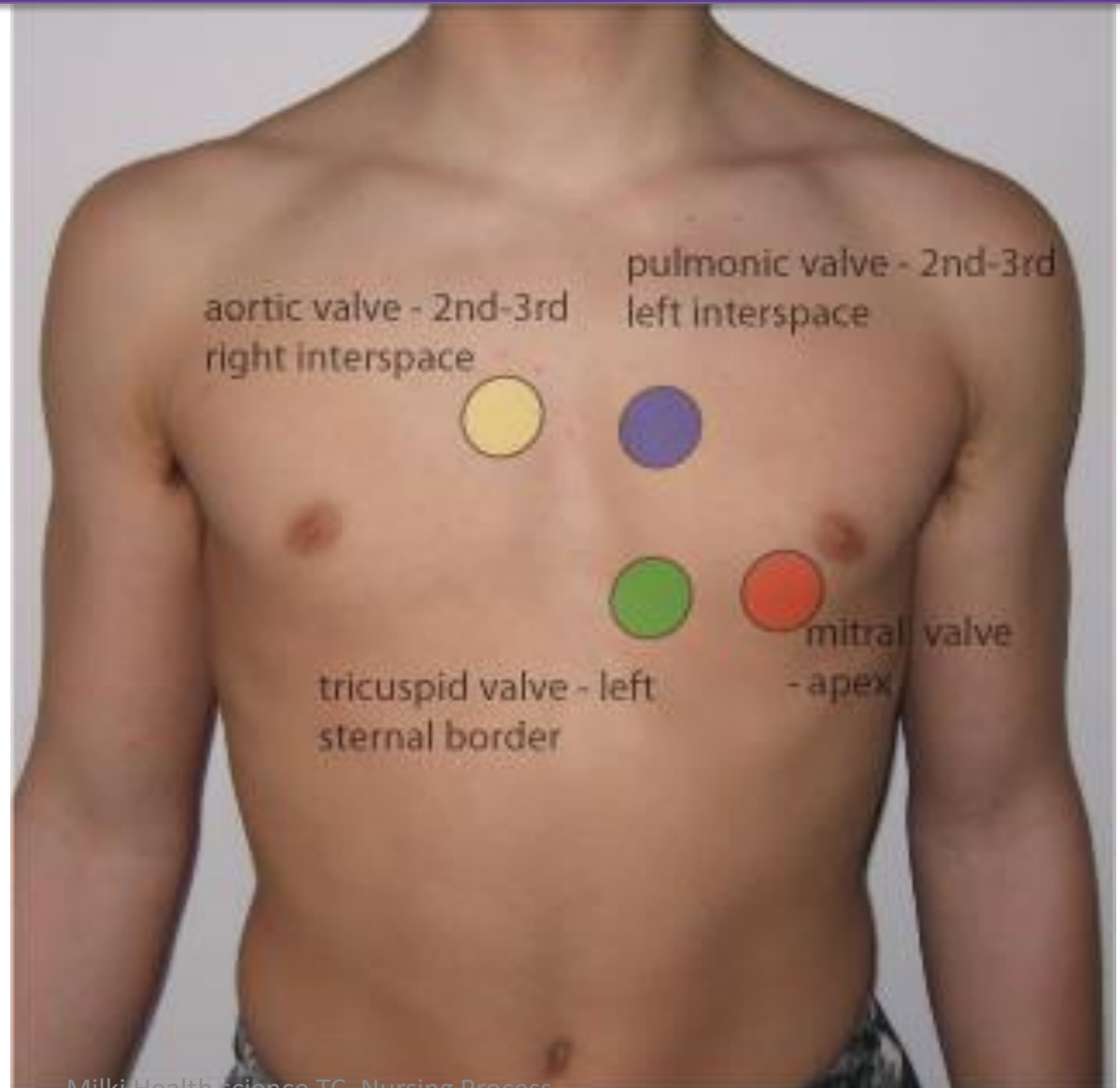


Sites for Auscultating the Abdomen

Site for auscultation the Heart valve

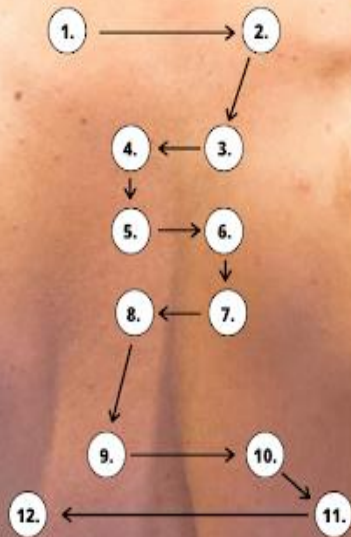


Site for auscultation the Heart valve

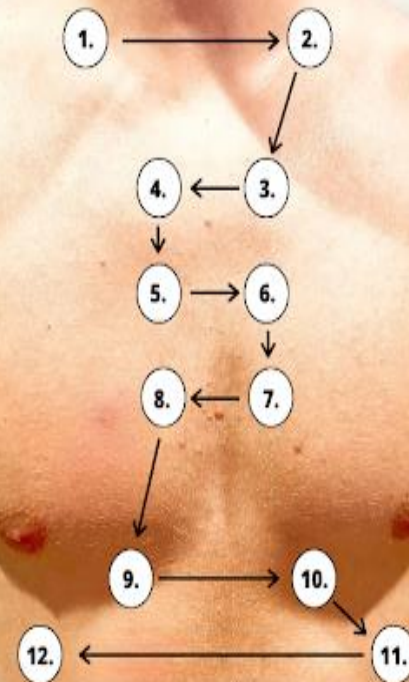


Auscultations of the Lung

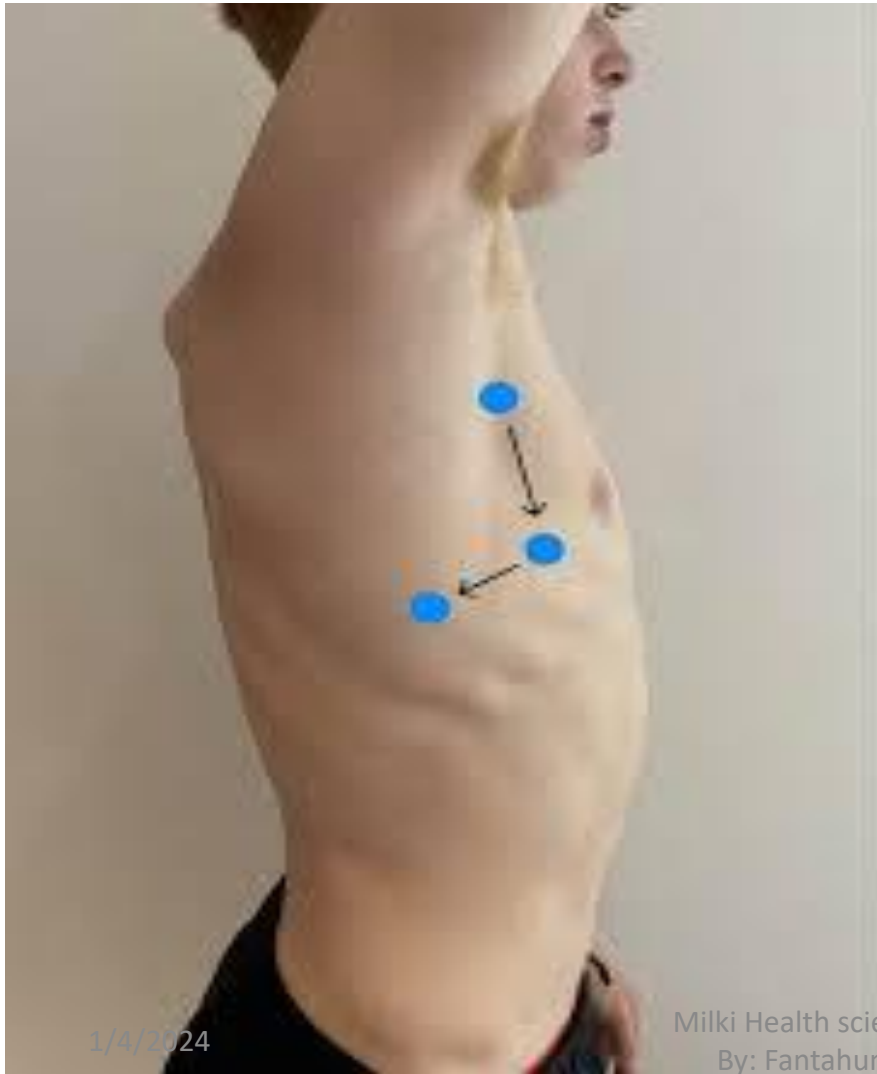
Respiratory
Auscultation Pattern
Posterior



Respiratory
Auscultation Pattern
Anterior



Auscultations of the Lung



GORDON'S FUNCTIONAL HEALTH PATTERNS

Gordon's functional health patterns

- **Gordon's functional health patterns** is a method devised by **Marjory Gordon** to be used by nurses in the nursing process to provide a more comprehensive nursing assessment of the patient.

MARJORY GORDON



11 Gordon's functional health patterns

1. Health Perception and health Management pattern
2. Nutritional metabolic Pattern:
3. Elimination Pattern
4. Activity exercise pattern
5. Sleep rest pattern

Gordon's functional ...Cont'd

6. Cognitive-perceptual pattern
7. Self perception/self concept pattern
8. Role relationship pattern
9. Sexuality reproductive pattern
10. Coping/stress tolerance pattern
11. Value-Belief Pattern

1. Health Perception and Health Management Pattern:

- What is your opinion about health?
- Are you immunized about seven target diseases?
- Last immunization?
- Do you have any allergy? If yes then type of allergy.
- Any surgery in past? What type of surgery?
- Last physical examination & for what purpose.
- Are you using any medicine recently?
- Do you know about these medicines?

(2) Nutrition and Metabolism Pattern

- ❖ Assessment is focused on the pattern of food and fluid consumption relative to metabolic need.
 - ☐ Ask about their skin, scalp and nails?
 - ☐ What is your diet menu?
 - ☐ Any food restriction regarding disease point of view?
 - ☐ Any food restriction regarding religious point of view?
 - ☐ Any food like or dislike?
 - ☐ Any food allergy?

(3) Elimination Pattern:

- Data collection is focused on **excretory patterns (bowel, bladder, skin).**
- Excretory problems such as
 - ✓ Incontinence,
 - ✓ Constipation,
 - ✓ Diarrhea, and
 - ✓ Urinary retention.



(3) Elimination Pattern...Cont'd

☐Urine:

- Color of urine, amount, frequency, odor and any discharge.
- Any urinary problem, dysurea, Anurea, Oligourea, , polyuria.

☐Defecation:

- Are you using any laxative? If yes which?
- Any problem during passing defecation?

(4) Activity and Exercise Pattern:

- ☐ Do you have any breathing problem?
- ☐ Do you have cough? (Productive or non productive)
- ☐ Any changes in heart beat during exercise
- ☐ What type of exercise you do or any problem during exercise



(5) Cognition and Perception Pattern:

- Assessment is focused on the ability to thinking, decision making, and problem solving.
- Orientation about
 - ✓ **Time**
 - ✓ **Place and**
 - ✓ **Person.**
- Any difficulty in sentence making?
- Loss of memory.



(6) Sleep and rest pattern:

- Dysfunctional sleep patterns, fatigue, and responses to sleep deprivation may be identified.
 - ✓ Sleeping hour?
 - ✓ Are you using nap (evening type sleeping).
 - ✓ What do you feel after waking? (Fresh, headache, drowsy).
 - ✓ Are you using any medication for sleeping?
 - ✓ Do you have any exercise or walking at night?



(7) Self-Perception and self-concept pattern:

- Assessment is focused on the person's attitudes toward self, including: Body image, and Sense of self-worth.
- What is your self perception about yourself?
- Are you satisfied with your self body image?
- Do you like grooming?



(8) Roles and relationships pattern:

- Assessment is focused on the person's roles in the family and relationships with others.
 - ✓What is your role in family?
 - ✓If you are in hospital then who will perform your responsibilities?
 - ✓All the family members are cooperative with you?
 - ✓Who is decision maker in your family?



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(9) Sexuality and Reproduction Pattern

- Assessment is focused on the person's satisfaction or dissatisfaction with sexuality patterns and reproductive functions.
 - ✓ Do you have any sexual problem? (loss of libido)
 - ✓ Any erectile dysfunction
 - ✓ Infertility



(10) Coping and stress tolerance pattern:

- Assessment is focused on the person's perception of stress and on his or her coping strategies Support systems are evaluated, and symptoms of stress are noted.
- If you have stress then what is your coping mechanism towards stress?
 - Crying, angry, violence, (what is your opinion regarding that)



(11) Values and Belief Pattern

- Assessment is focused on the person's values and beliefs (including spiritual beliefs).
 - ✓ What is your religion?
 - ✓ Do you offer prayer?



Components that should be written in Assessment

1. Biographic data
2. Chief Complaint
3. History of present illness
4. Past Medical History
5. Past surgical History
6. Family history
7. Social History
- 8. Review of Systems/11 functional pattern assessment**
9. Physical Examination
 1. General appearance
 2. Head to toes P/E
 3. Vital sign
 4. Laboratory Investigation done.

History of present illness (HPI)

- The HPI is a **chronological description** of the development of the patient's present illness from the **first sign and/or symptom or from the previous encounter to the present.**

History of present illness (HPI)

- This 23-year-old male patient arrived at the health facility with complaints of watery diarrhea, a persistent episode of vomiting, sunken eyes, fatigue, and weakness. These symptoms began three days prior to his arrival. He is currently taking Ciprofloxacin 500mg orally twice daily, but there has been no improvement in his condition. The vomiting is relieved when he abstains from eating, but worsens when he eats food.

Taken from Exit Exam question

- A 25 year- old male with known liver disease presents with cough, fever and weight loss for one month sputum examination for acid fast bacilli is positive 3 times.
- Which combination anti tuberculosis treatment is most appropriate for this patient?
 - A. 2ERHZ/4HE
 - B. 2RHZ/4RH
 - C. 2SERH/6RH
 - D. 2SRZE/6HE

2. Nursing Diagnosis

...to be continued





Pre-test



- A 37 year's old female patient came to your hospital with complaint of Nausea and Vomiting, Bloating and Chronic pelvic pain and heavy bleeding during periods or between periods and on Laparoscopic examination there is presence of endometrial tissue outside of the uterus. Based on this scenario what is your problem focused Nursing diagnosis?
 - A. Acute Pain related to the chronic pelvic pain experienced by the patient,
 - B. Risk for Anxiety related to the uncertainty and discomfort
 - C. Risk for Infection related to any surgical interventions that may be necessary to treat the endometriosis.
 - D. Endometriosis

Pre-test



- A 30-year-old female patient presents with menstrual irregularities, a history of breast cancer, and ongoing chemotherapy treatment. She underwent repeated imaging studies of her pelvic region one year ago. Based on these findings, what is your Nursing diagnosis?
- A. Primary ovarian insufficiency B. Asherman's syndrome
C. Polycystic ovary syndrome D. Endometriosis E. none

Taken from Exit Exam question



- 63. A patient has “Fluid volume deficit related to persistent nausea and vomiting as evidenced by Bp=70/40 mmHg”. What type of nursing diagnosis is it?
 - A. Actual nursing diagnosis
 - B. Potential nursing diagnosis
 - C. Syndromic nursing diagnosis
 - D. Well nursing diagnosis

Taken from Exit Exam question



64. A patient has “Fluid volume deficit related to persistent nausea and vomiting as evidenced by Bp=70/40 mmHg”. Which part of the statement is the etiology?

- A. Bp=70/40mmHg
- B. Fluid volume deficit
- C. Persistent nausea vomiting
- D. Persistent nausea vomiting evidenced by BP

Taken from Exit Exam question



- A 55 year-old man is a known hypertensive patient. Blood pressure measurement of the patient revealed 190/100mmHg. The treating physician initiated patient with furosemide 40mg twice a day to control the increased blood pressure.
- What would be the most possible **potential nursing diagnosis** to this patient?
 - A. Risk for decreased serum K⁺ levels related to released potassium
 - B. Hypotension related to increased urine flow
 - C. Risk for increased blood glucose levels related to increased metabolism
 - D. Fatigue related to nutrition less than body requirement

Exit Exam/COC question & related topics

- What is your possible diagnosis?
- What is your diagnosis?
- What is your medical diagnosis?
- What is your priority nursing intervention

2. Nursing Diagnosis



Nursing Diagnosis

- A nursing diagnosis is a clinical judgment concerning a an undesirable human response to health conditions/life processes, or a vulnerability for that response.
- Nursing diagnoses are the foundation of care plans.
- Involves further **analysis** and **synthesis** of the data that have been collected.

Nursing diagnoses...Cont'd

- The purpose of nursing diagnosis is to **identify actual or potential health problems** that can be addressed through nursing interventions.
- Nursing diagnosis helps nurses to:
 - A. Identify patient health problems
 - B. Develop a care plan
 - C. Communicate with other healthcare professionals
 - D. Monitor patient progress
 - E. Evaluate the effectiveness of nursing interventions:

Nursing diagnoses...Cont'd

- Nursing diagnoses drive actions and allow for continued
 - ✓ Assessment,
 - ✓ Prioritization,
 - ✓ Organization, and
 - ✓ Effective health outcomes.

Nursing Diagnosis...Cont'd

- According to the North American Nursing Diagnosis Association (NANDA) a **nursing diagnosis**.
- Is a clinical judgment about individual, family, or community responses to **actual** or **potential health** problems / life processes.
- ***Nursing diagnosis*** is care focused.

Nursing Diagnosis...cont'd

- Clients receive both **medical** and **nursing** diagnoses.
- A nursing diagnosis is a diagnosis that is based upon the **response** of the patient to the medical condition.
- This is why it is called a '*nursing diagnosis*' because these are things that have a specific action that is related to what nurses have autonomy to take action about.

Nursing Diagnosis...cont'd

- Nurses treat the patient with everything that is related to human response to a specific disease.
- This includes anything that is a
 - ✓ *Physical*,
 - ✓ *Mental*, and
 - ✓ *Spiritual* type of response.
- A nursing diagnosis is initiated by a nurse and describes a response to the medical diagnosis.



NANDA International



NANDA International



- NANDA is an acronym for the **North American Nursing Diagnosis Association**.
- It was founded in 1982 by nursing professionals who wanted to develop a standardized language for nursing diagnoses that could be used across different healthcare settings.
- The association was later renamed to NANDA International to reflect its global reach.



NANDA International...Cont'd

- NANDA International is a professional organization that focuses on the
 - ✓ Development,
 - ✓ Refinement, and
 - ✓ Dissemination of standardized nursing diagnoses.
- NANDA International is an organization that supports the use and development of *standardized nursing terminology throughout clinical settings.*

Purpose and Mission of NANDA-I

- This organization's mission is:
 - To use nursing diagnoses to promote high-quality patient care through evidence-based care and consistent terminology.
 - We fund research through the NANDA-I Foundation.
 - We are a supportive and energetic global network of nurses who are committed to improving the quality of nursing care through evidence-based practice.



Diagnostic Axes

□ Axis

- An axis is operationally defined as a dimension of the human response that is considered in the diagnostic process.
- There are **seven axes** which parallel the International Standards Reference Model for a Nursing Diagnosis.
 - **Axis 1:** the diagnostic focus
 - **Axis 2:** subject of the diagnosis (individual, caregiver, family, group, community)



Diagnostic Axes...Cont'd

- **Axis 3:** Judgment (impaired, ineffective, etc.)
- **Axis 4:** Location (bladder, auditory, cerebral, etc.)
- **Axis 5:** Age (infant, child, adult, etc.)
- **Axis 6:** Time (chronic, acute, intermittent)
- **Axis 7:** status of the diagnosis (problem-focused, risk, health promotion).



Diagnostic Axes

- **Definitions of the Axes**

- **Axis 1 The Diagnostic Focus**

- The diagnostic focus is the principal element or the fundamental and essential part, the root, of the diagnostic concept.
- It describes the “**human response**” that is the core of the diagnosis.



Diagnostic Axes...Cont'd

❑ Axis 2: Subject of the Diagnosis

- The person(s) for whom a nursing diagnosis is determined.
- The values in Axis 2 which represent the NANDA-I definition of “patient” are:
 - ✓ **Individual:** a single human being distinct from others, a person.
 - ✓ **Caregiver:** a family member or helper who regularly looks after a child or a sick, elderly, or disabled person.



Diagnostic Axes...Cont'd

- ✓ **Family:** two or more people having continuous or sustained relationships, sensing common meaning, and sharing certain obligations toward others; related by blood and/or choice.
- ✓ **Group:** a number of people with shared characteristics.
- ✓ **Community:** a group of people living in the same locale under the same governance. Examples include neighborhoods and cities.



Diagnostic Axes...Cont'd

❑ Axis 3 Judgment

- A descriptor or modifier that limits or specifies the meaning of the diagnostic focus.
- The diagnostic focus together with the nurse's judgment about it forms the diagnosis.



Diagnostic Axes...Cont'd

❑ **Axis 4 Location**

- Describes the parts/regions of the body and/or their related functions – all
 - ✓ Tissues,
 - ✓ Organs,
 - ✓ Anatomical sites, or
 - ✓ Structures.

❑ Axis 5 Age

- Refers to the age of the person who is the subject of the diagnosis (**Axis 2**).
- The values in Axis 5 are noted below, with all definitions except that of older adult being drawn from the World Health Organization (2013):
 - ✓ **Fetus:** an unborn human more than eight weeks after conception, until birth.
 - ✓ **Neonate:** a child < 28 days of age
 - ✓ **Infant:** a child > 28 days and < 1 year of age



Diagnostic Axes...Cont'd

- ✓ **Child:** person aged 1 to 9 years, inclusive
- ✓ **Adolescent:** person aged 10 to 19 years, inclusive
- ✓ **Adult:** a person older than 19 years of age unless national law defines a person as being an adult at an earlier age
- ✓ **Older adult:** a person > 65 years of age



❑ Axis 6 Time

- Describes the duration of the diagnostic concept (Axis 1:
 - **Acute:** lasting <3 months
 - **Chronic:** lasting >3 months
 - **Intermittent:** stopping or starting again at intervals, periodic, cyclic
 - **Continuous:** uninterrupted, going on without stop.



❑ **Axis 7 Status of the Diagnosis**

- Refers to the actuality or potentiality of the problem/syndrome or to the categorization of the diagnosis as a health promotion diagnosis.
- The values in Axis 7 are:
 1. Problem-focused,
 2. Health Promotion,
 3. Risk, and
 4. Syndrome.

Nursing Diagnoses Vs. Medical Diagnoses



Nursing Diagnoses Vs Medical Diagnoses

- ❑ **Medical diagnoses** focus on diseases or other medical problems that have been identified by the
 - ✓ Physician, or
 - ✓ Physician's assistant,

Nursing Diagnoses Vs Medical Diagnoses

- Nursing diagnoses focus on the *human response* to health conditions and life processes and are made independently by **RNs**.
- Patients with the same medical diagnosis will often *respond* differently to that diagnosis and thus have different nursing diagnoses.

Nursing Diagnoses Vs Medical Diagnoses

- **For example**, Two patients have the same medical diagnosis of heart failure.
- However, one patient may be interested in learning more information about the condition and the medications used to treat it,
- Whereas another patient may be experiencing anxiety when thinking about the effects this medical diagnosis will have on their family.

Nursing Diagnoses Vs Medical Diagnoses

- ❑ **Nursing diagnoses** consider the patient's and family's
 - ✓ Needs,
 - ✓ Attitudes,
 - ✓ Strengths,
 - ✓ Challenges, and
 - ✓ Resources as a customized nursing care plan is created to provide **holistic** and individualized care for each patient.

Nursing Diagnoses Vs Medical Diagnoses...Cont'd

- **Medical diagnoses** focus on the
 - ✓ Disease or
 - ✓ Pathology that affects the patient.
- In contrast, **nursing diagnoses** concentrate on the
 - ✓ Patient's response to the illness or
 - ✓ Life circumstance,
 - ✓ Which can be either a **physiological** or **psychological** response.

Examples of how medical and nursing diagnoses can work together

□ Example 1:

- A patient with a medical diagnosis of cerebrovascular accident (stroke) may lead to the complementary nursing diagnosis of **unilateral neglect**.
- Without the medical diagnosis, the nurse would not know what was causing **unilateral neglect**.

❑ Example 2:

- With the medical diagnosis of **Dehydration**, the nurse knows that the patient is experiencing **deficient fluid volume** (nursing diagnosis).
- Therefore, the nurse may implement the interventions of administering IV fluids and recording intake and output for this patient.
- Nursing and medical diagnoses are complementary and guide each other to create a **holistic clinical story**.

Nursing Diagnoses Vs Medical Diagnoses...Cont'd

- On the other hand, **a medical diagnosis** is made by the
- physician or
- Advanced health care practitioner that deals more with the
 - ✓ Disease,
 - ✓ Medical condition, or
 - ✓ Pathological state only a practitioner can treat.

Nursing Diagnoses Vs Medical Diagnoses...Cont'd

- Examples of medical diagnoses are
 - ✓ Diabetes Mellitus,
 - ✓ Tuberculosis,
 - ✓ Amputation,
 - ✓ Hepatitis, and
 - ✓ Chronic Kidney Disease.
- The medical diagnosis normally does not change.

Nursing Diagnoses Vs Medical Diagnoses...Cont'd

- The main differences between nursing diagnosis and medical diagnosis are:

1. Purpose:

- ❑ **Medical diagnosis** is used to identify and treat specific diseases or conditions,
- ❑ while nursing diagnosis is used to identify patient needs that can be addressed through nursing interventions.

2. Approach:

- **Medical diagnosis** is based on medical knowledge and diagnostic tests,
- While **nursing diagnosis** is based on patient assessment data and nursing knowledge.

3. Scope:

- **Medical diagnosis** focuses on the disease or condition itself,
- **While nursing diagnosis** focuses on the patient's response to the disease or condition.

Medical vs. Nursing diagnosis

Medical diagnosis	Nursing diagnosis
Identifies conditions the MD is licensed & qualified to treat	Identifies situations the nurse is licensed & qualified to treat
Focuses on <u>illness</u> , injury or disease processes	Focuses on the clients <u>responses</u> to actual or potential health / life problems

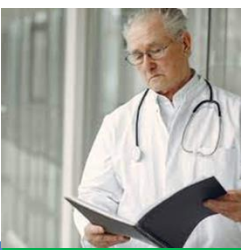
Ex: Diagnosis

Nursing diagnosis	Medical diagnosis
Breathing patterns, ineffective	Chronic obstructive pulmonary disease
Activity intolerance	Cerebrovascular accident
Pain	Appendectomy
Body image disturbance	Amputation
Body temperature, risk for altered	Strep throat

Medical Diagnosis



- Dr. T. Heather Herdman, CEO and Executive Director of NANDA simply states “**A medical diagnosis** is a concept that defines a disease process or injury” and “**nursing diagnoses** describe human response to potential or actual health problems”.
- The process of identifying a disease, condition, or injury from its signs and symptoms. A health history, physical exam, and tests, such as blood tests, imaging tests, and biopsies, may be used to help make a diagnosis.
- A medical diagnosis is a clinical judgment by the **physician** that identifies or determines a **specific disease, condition, or pathological state**.



Medical Diagnosis

Nursing Diagnosis

➤ Focus on the illness, injury, or disease process.

➤ Focuses on the responses to actual or potential health problems or life processes.

➤ Remains constant until a cure is effected.

➤ Changes as the client's response and/or the health problem changes.

➤ Identifies conditions the health care practitioner is licensed and qualified to treat.

➤ Identifies situations in which the nurse is licensed and qualified to intervene.

Comparison of Select Nursing and Medical Diagnoses

NURSING DIAGNOSIS	MEDICAL DIAGNOSIS
Ineffetive Airway Clearance	Pneumonia
Disturbed Body Image	Amputation
Risk for Unstable Blood Glucose	Diabetes Mellitus
Impaired Urinary Elimination	Post-Op Prostatectomy
Self-Care Deficit: Dressing & Grooming	Cerebrovascular Accident



Categories of Nursing Diagnosis

Categories of Nursing Diagnosis

- NANDA-I recognizes four categories of nursing diagnoses:
 1. Problem focused diagnosis,
 2. Risk diagnosis,
 3. Health promotion diagnosis, and
 4. Syndrome.



1. Problem focused diagnoses

1. Problem focused diagnoses

- Also known as **actual diagnoses**,
- Are patient issues or Problems that are present and observable during the assessment phase.
- They are based on the presence of certain signs or symptoms.



Problem-Focused Diagnosis...Cont'd

- A problem-focused nursing diagnosis contains all three components of the **PES format**:
 1. **Problem (P)** – statement of the patient response (nursing diagnosis)
 2. **Etiology (E)** – related factors contributing to the nursing diagnosis
 3. **Signs and Symptoms (S)** – defining characteristics manifested by that patient



How to Write a Nursing Diagnosis



3-Part Nursing Diagnosis (P-E-S Format)

- Problem (Label) + Etiology (Cause or Related Factors) + Signs/Symptoms (Defining Characteristics)
- **Problem**-Focused Diagnosis_____ **related to** _____(Etiology or Related Factors) **as evidenced by** _____(Defining Characteristics).



Problem focused diagnoses...Cont'd

- **Example:1 Deficient fluid volume** related to prolonged vomiting as **evidenced** by increased pulse rate, increased urine concentration, and poor skin turgor.
- **Example: 2 Acute pain** related to surgical incision as **evidenced** by patient reports of pain and guarded movement.



Problem focused diagnoses...Cont'd

- **Example 3:** Impaired skin integrity **related** to prolonged bed rest as **evidenced** by pressure ulcer on the sacrum.
- **Example 4:** Ineffective breathing pattern related to COPD exacerbation as evidenced by shortness of breath and use of accessory muscles.



Problem focused diagnoses...Cont'd

- **Example 5:** Impaired mobility **related** to lower extremity weakness as **evidenced** by difficulty ambulating and requiring assistance with transfers
- **Example 6:** Impaired urinary elimination **related** to urinary retention as **evidenced** by decreased urine output and bladder distention..



2. Risk Nursing diagnosis

2. Risk Nursing diagnosis

- A clinical judgment concerning the vulnerability of an
 - ✓ Individual,
 - ✓ Family,
 - ✓ Group, or
 - ✓ Community for developing an **undesirable human response** to health conditions/life processes.



2. Risk Nursing diagnosis...Cont'd

- A risk nursing diagnosis should be supported by evidence of the patient's risk factors for developing that problem.
- Different experts recommend different phrasing. NANDA-I 2018-2020 recommends using the phrase **“as evidenced by”** to refer to the **risk factors** for developing that problem.



2. Risk Nursing diagnosis...Cont'd

- A risk diagnosis consists of the following:
 - ❑ **Problem (P)** – statement of the patient response (nursing diagnosis)
 - ❑ **As Evidenced By** – Risk factors for developing the problem

2-Part Nursing Diagnoses



- **2- Part Nursing Diagnoses = Label + Risk Factors**
- Risk focused diagnosis label _____ as evidenced by _____ (Risk Factors).
- **Defining characteristics are not possible because the problem isn't “actual” yet.**

2-Part Nursing Diagnoses...



❑ Examples:

- **Risk** for infection **as evidenced** by a history of cancer and recent surgery.
- **Risk** for falls **as evidenced** by a history of falls, use of an assistive device, and visual difficulties.

Example of potential Nursing diagnosis



- **Risk for Falls as evidenced** by old age and use of walker.
- **Risk for falls as evidenced** by muscle weakness
- **Risk for Injury as evidenced** by altered mobility
- **Risk for Infection as evidenced** by immunosuppression
- **Risk for Infection as evidenced** by break in skin integrity

Pretest



1. What is the top priority nursing intervention for the following nursing diagnosis?
 - A. Fluid Volume Excess,
 - B. Enhanced Readiness for Health Promotion,
 - C. Risk for Falls
 - D. Risk for Frail Elderly Syndrome.



2) A -32- year old known RVI patient who has diarrhea for the past 1month with decrease intake came to the ED with weakness and change in mentation of 1 day duration. On physical examination, Bp- un recordable, PR 140 b/m, RR 36 b/m, T° 35°C, Oxygen saturation- 80%. What is your priority management for this patient?

- A. Check his airway, breathing and secure wide bore bilateral IV line
- B. Secure bilateral IV line
- C. Start warming up the patient because he is hypothermic
- D. Take further history regarding the duration and the quality of the diarrhea

3) A -20-year old male patient who sustained car accident came to you with loss of consciousness of 5 hours duration. On P/E, the patient is snoring with excessive secretions from his mouth. What will be the priority nursing management for this patient?

- A. Take vital signs
- B. Assess for presence of bleeding
- C. Open airway and apply suctioning
- D. Secure bilateral IV lines to manage shock

4) What is the priority nursing intervention for a patient who has hypotension secondary to diarrhea and vomiting?

- A. Give medication
- B. Identify underline cause
- C. Resuscitate with fluid
- D. Taking detailed history



11. An 18-day-old neonate admitted in neonatal intensive care unit for the treatment of neonatal tetanus with a clinical feature of unable to breast feeding, spasm, stiffness, accumulation of oral secretion and seizure.

What is the most likely priority nursing intervention for the neonate?

- Administration of diazepam to the spasm
- Insert NG- tube for express breast feeding
- Secure IV line for maintenance fluid
- Suction the secretion to prevent aspiration ✓





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8. A 20 weeks pregnant woman came with the complaint of vaginal bleeding for three hours. She had abdominal and back pain. On examination, her hemoglobin level was 9g/dl (NR: 10.5-13.7 g/dl), blood pressure 90/50mmHg and pulse rate 110/min. The woman was diagnosed for inevitable abortion.

What is the priority nursing intervention for this woman?

- (A) Provide analgesia
- (B) Reassure the woman
- (C) Prepare for blood transfusion
- (D) Resuscitate with normal saline



34. A child was admitted to pediatric ward for the treatment of femoral bone fracture. The child was on traction with a cast and complained severe unpleasant sensation at the fracture site.

What is priority nursing diagnosis for this case?

(A) Acute pain

(B) Impaired mobility

(C) Activity intolerance

(D) Impaired activities of daily living



- Which of the following is **wellness Nursing diagnosis**?
 - A. Risk for Infection as evidenced by immunosuppression
 - B. Readiness for enhanced breastfeeding as evidenced by the patient stating their desire to exclusively breastfeed.
 - C. Post-trauma syndrome as evidenced by insomnia and risk for suicide.
 - D. Ineffective breathing pattern related to COPD exacerbation as evidenced by shortness of breath and use of accessory muscles.



- Which of the following is **Collaborative Nursing diagnosis**?
 - A. Risk for Infection as evidenced by immunosuppression
 - B. Readiness for enhanced breastfeeding as evidenced by the patient stating their desire to exclusively breastfeed.
 - C. Post-trauma syndrome as evidenced by insomnia and risk for suicide.
 - D. Ineffective breathing pattern related to COPD exacerbation as evidenced by shortness of breath and use of accessory muscles.

Necessary Topics, that you should know

- ☐ Possible diagnosis
- ☐ Diagnosis
- ☐ Medical diagnosis
- ☐ Priority Actual Nursing diagnosis
- ☐ Priority Nursing intervention



3. Health promotion diagnosis

3. Health promotion diagnosis

- **Health promotion diagnosis**, or wellness diagnosis, are the clinical judgments about the motivation and desire to increase well-being and reach one's health potential.
- These judgments express **a patient's readiness** to improve specific health behaviors.



Health-promotion Nursing Diagnosis...Cont'd

- Health promotion diagnosis can exist at:
 - ✓ An individual,
 - ✓ Family,
 - ✓ Group, or
 - ✓ Community level.



Health-promotion Nursing Diagnosis...Cont'd

- In order to make a health-promotion diagnosis, the following must be present:
 - Defining characteristics which begin with the phrase, “**Expresses desire to enhance...**”.

Writing a Health Promotion-Based Diagnosis

- A health promotion diagnosis is also described using a two-part statement.

□ 2-Part Nursing Diagnoses

- 2-Part Nursing Diagnosis =
 1. Label
 2. Defining Characteristics

□ **Health promotion label _____ as evidenced by _____ (Defining characteristics).**

Writing a Health Promotion-Based Diagnosis

❑ Examples:

- **Readiness for** enhanced breastfeeding as **evidenced** by the patient stating their desire to exclusively breastfeed and requesting information on how to achieve this.



Writing a Health Promotion-Based Diagnosis...Cont'd

- Readiness for enhanced spiritual well-being.
- Readiness for enhanced coping.
- Readiness for enhanced self-care.
- Readiness for enhanced parenting.
- Readiness for enhanced exercise engagement.



NANDA International

4. Syndromes Nursing diagnosis

4. Syndromes Nursing diagnosis

- **Syndromes** are the least present diagnosis in the NANDA-I taxonomy.
- They concern the clinical judgments that relate to a **cluster of nursing diagnoses** that occur together and are dealt with through similar interventions.



4. Syndrome...Cont'd

- In order to make a **syndrome diagnosis**, the following must be present:
 - Two or more nursing diagnoses must be used as defining characteristics.
 - **Related factors** may be used if they add clarity to the definition, but are not required.



4. Syndrome...Cont'd

- A syndrome statement consists of these items:
 - ✓ **Problem (P)** – the syndrome
 - ✓ **Signs and Symptoms (S)** – the defining characteristics are two or more similar nursing diagnoses

Writing a Syndrome Diagnosis

- **2-Part Nursing Diagnoses**

- ✓ Label

- ✓ Nursing Diagnoses that make up the syndrome

□ E.g

- Syndrome diagnosis label _____ as evidenced by _____ (2 or more nursing diagnoses that support the syndrome diagnosis).

Writing a Syndrome Diagnosis

□ Examples:

- ✓ Post-trauma **syndrome as evidenced** by disturbed sleep pattern and ineffective relationship functioning.
- ✓ Post-trauma **syndrome as evidenced** by insomnia and risk for suicide.
- ✓ Post-traumatic stress **syndrome related** to experiencing a traumatic event as evidenced by flashbacks, nightmares, and avoidance behaviors.

Writing a Syndrome Diagnosis

- ✓ Chronic fatigue **syndrome related** to prolonged fatigue as evidenced by exhaustion, weakness, and difficulty concentrating.
- ✓ Irritable bowel **syndrome related** to gastrointestinal dysfunction as evidenced by abdominal pain, bloating, and altered bowel habits.

3. Planning



What is nursing planning in nursing process?

- Once the treatment team agrees on the diagnosis, you will plan a course of treatment that takes into account short- and long-term goals
- The planning stage is where **goals** and **outcomes** are formulated that directly impact patient care.
- These patient-specific goals and the attainment of such assist in ensuring a positive outcome.
- Nursing care plans are essential in this phase of goal setting.



3. Planning...Cont'd

- Once the diagnoses have been established, the nurse develops a plan of care that includes
 - ✓ Specific goals,
 - ✓ Interventions, and
 - ✓ Expected outcomes.
- The plan of care should be individualized to meet the patient's unique needs and preferences.

The planning phase involves several tasks

1. Establishing priorities of nursing diagnoses.
2. Setting goals and developing expected outcomes (outcome identification).
3. Planning nursing interventions (with collaboration and consultation as needed).
4. Record the entire nursing care plan in the client record.

☐ Those diagnoses involving life-threatening situations are given the highest priority.



3. Planning...Cont'd

- **Expected outcomes:** are specific objectives related to the goals and are used to evaluate the nursing interventions.
- They must be measurable, have a time limit, and be realistic.



3. Planning...Cont'd

- Effective nursing planning requires collaboration with other healthcare professionals, including
 - ✓Physicians,
 - ✓Therapists, and
 - ✓Social workers.
- It also requires ongoing reassessment and modification as the patient's condition changes.



3. Planning...Cont'd

- By developing a comprehensive plan of care that addresses the **patient's unique needs and goals**, nurses can help to improve **patient outcomes** and promote overall health and well-being.



3. Planning...Cont'd

- **Goals should be:**
 - ☐ Specific
 - ☐ Measurable or Meaningful
 - ☐ Attainable or Action-Oriented
 - ☐ Realistic or Results-Oriented
 - ☐ Timely or Time-Oriented

3. *Planning...Cont'd*

❑ Nursing Diagnosis:

- Deficient knowledge about the therapeutic regimen for lifelong thyroid replacement therapy

❑ Goal:

- ✓ Knowledge and acceptance of the prescribed therapeutic regimen



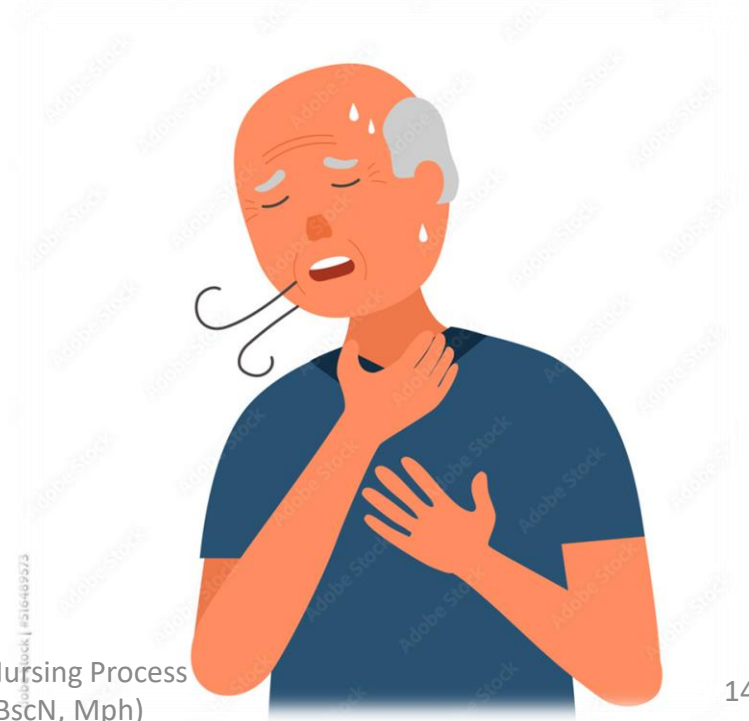
3. *Planning...Cont'd*

- ❑ **Nursing Diagnosis:** Risk for imbalanced body temperature
- ❑ **Goal:** Maintenance of normal body temperature



3. *Planning...Cont'd*

- ❑ **Nursing Diagnosis:** Ineffective breathing pattern related to depressed ventilation
- ❑ **Goal:** Improved respiratory status and maintenance of normal breathing pattern.



4. Nursing Interventions





4. Nursing Interventions

- **Nursing interventions** are activities or actions that a nurse performs to achieve client goals.
- Interventions chosen should focus on eliminating or reducing the etiology of the **priority nursing problem** or diagnosis.
- As for risk nursing problems, interventions should focus on reducing the client's risk factors.



Nursing Interventions...Cont'd

- In this step, nursing interventions are identified and written during the planning step of the **Nursing Process**;
- However, they are actually performed during the implementation step.
- The nurse must continue to assess the client's condition **before, during, and after** the nursing intervention.

Nursing implementation activities include

1. Ongoing assessment.
2. Establishment of priorities.
3. Allocation of resources.
4. Initiation of nursing interventions.
5. Documentation of **interventions** and **client response**.

Types of Nursing Interventions

- 1. Independent,**
- 2. Dependent, or**
- 3. Collaborative:**



1. Independent nursing interventions

- Independent nursing interventions are activities that nurses are licensed to initiate based on their sound judgement and skills. **Includes:**
 - ✓ Ongoing assessment,
 - ✓ Emotional support,
 - ✓ Providing comfort,
 - ✓ Teaching,
 - ✓ Physical care, and
 - ✓ Making referrals to other health care professionals.

2) Dependent nursing interventions

- Dependent nursing interventions are activities carried out **under the physician's orders or supervision.**
- **Includes:**
 - ✓ Orders to direct the nurse to provide medications,
 - ✓ Intravenous therapy,
 - ✓ Diagnostic tests,
 - ✓ Treatments,
 - ✓ Diet, and
 - ✓ Activity.
 - ✓ Assessment and providing explanation while administering medical orders.

3) Collaborative interventions

- Collaborative interventions are actions that the nurse carries out in collaboration with other health team members, such as
 - ✓Physicians,
 - ✓Social workers,
 - ✓Dietitians, and
 - ✓Therapists.
- These actions are developed in consultation with other health care professionals to gain their professional viewpoint.

Nursing interventions should be:

- Safe and appropriate for the client's age, health, and condition.
- Achievable with the resources and time available.
- Inline with the client's values, culture, and beliefs.
- Inline with other therapies.
- Based on nursing knowledge and experience or knowledge from relevant sciences.

When writing nursing interventions, follow these tips:

- Write the date and sign the plan.
- The date the plan is written is essential for
 - ✓ Evaluation,
 - ✓ Review, and
 - ✓ Future planning.
- The nurse's signature demonstrates accountability.
- Nursing interventions should be specific and clearly stated.
- Use only abbreviations accepted by the institution.

Examples of nursing interventions may include:

- 1. Medication administration:** This may involve administering medications to manage pain, reduce inflammation, or treat infections.
- 2. Wound care:** This may involve cleaning and dressing wounds, monitoring for signs of infection, and providing education on wound care.

Examples of nursing interventions may include:

- 3) **Patient education:** This may involve providing information on healthy lifestyle choices, disease management, and medication management.
- 4) **Mobilization and activity promotion:** This may involve assisting patients with mobility and promoting physical activity to prevent complications such as bedsores and blood clots.

Examples of nursing interventions may include:

- 5) **Nutrition support:** This may involve assessing the patient's nutritional status, providing dietary recommendations, and monitoring for malnutrition.
- 6) **Psychological support:** This may involve providing emotional support, counseling, and referrals to mental health professionals.



Priority Nursing Interventions





Priority nursing interventions

- Priority nursing interventions are those that are considered most urgent or important in addressing the patient's health problems.
- These interventions are determined based on the
 - ✓ Severity of the patient's condition,
 - ✓ Potential risks and complications, and
 - ✓ Patient's goals and preferences.





Setting Priorities

- Setting priorities involves establishing a preferential sequence for addressing nursing diagnoses and interventions.
- In this step, the nurse and the client begin planning which of the identified problems requires attention first.
- Diagnoses can be ranked and grouped as having a
 - ✓ High,
 - ✓ Medium, or
 - ✓ Low priority.
- Life-threatening problems should be given high priority.

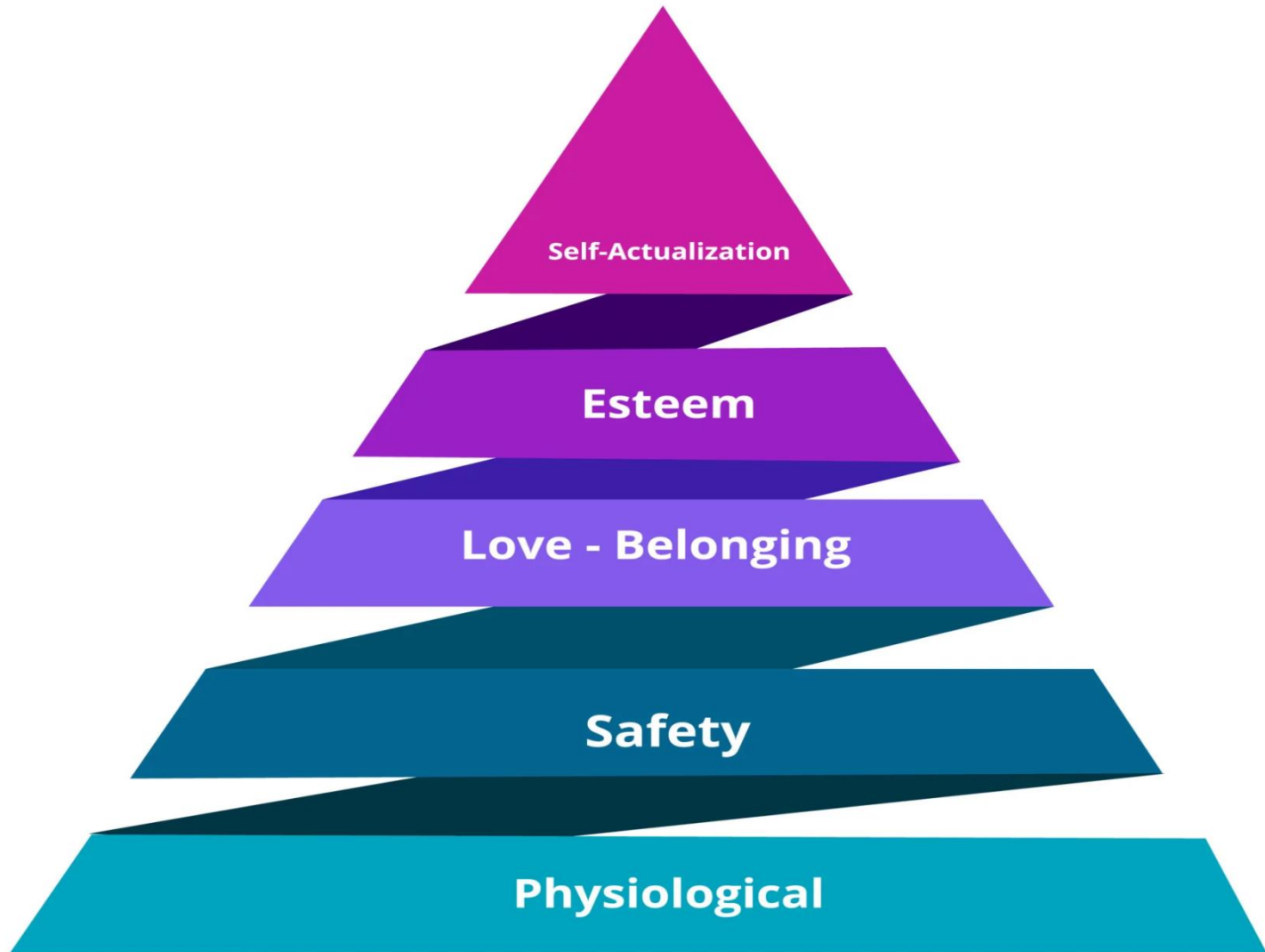
Priority nursing interventions criteria

1. Maslow's Hierarchy of Needs
2. ABCs
3. CURE
4. Data Cues





Maslow's Hierarchy of Needs



Maslow's Hierarchy of Needs

1. Basic Physiological Needs:

- ✓ Nutrition (water and food),
- ✓ Elimination (Toileting),
- ✓ Airway (suction)
- ✓ Breathing (oxygen)
- ✓ Circulation (pulse, cardiac monitor, blood pressure) (ABCs),
- ✓ Sleep,
- ✓ Sex,
- ✓ Shelter, and
- ✓ exercise.

Maslow's Hierarchy of Needs...Cont'd

2) Safety and Security:

- ✓ Injury prevention:
 - ✓ Side rails,
 - ✓ Call lights,
 - ✓ Hand hygiene,
 - ✓ Isolation,
 - ✓ Suicide precautions,





Maslow's Hierarchy of Needs...Cont'd

Safety and Security...Cont'd

- ✓ Fall precautions,
- ✓ Car seats,
- ✓ Helmets,
- ✓ Seat belts
- ✓ Fostering a climate of trust and safety (therapeutic relationship),
- ✓ Patient education (modifiable risk factors for stroke, heart disease).

Maslow's Hierarchy of Needs...Cont'd

3) Love and Belonging:

- ✓ Foster supportive relationships,
- ✓ Methods to avoid social isolation (bullying),
- ✓ Employ active listening techniques,
- ✓ Therapeutic communication, and
- ✓ Sexual intimacy.



Maslow's Hierarchy of Needs...Cont'd

4. Self-esteem:

- ✓ Acceptance in the community,
- ✓ Workforce,
- ✓ Personal achievement,
- ✓ Sense of control or empowerment,
- ✓ Accepting one's physical appearance or
- ✓ Body habitus.





Maslow's Hierarchy of Needs...Cont'd

5) Self-actualization:

- ✓ Empowering environment,
- ✓ Spiritual growth,
- ✓ Ability to recognize the point of view of others,
- ✓ Reaching one's maximum potential.



Maslow's Hierarchy



2. ABCs



Airway



Breathing



Circulation

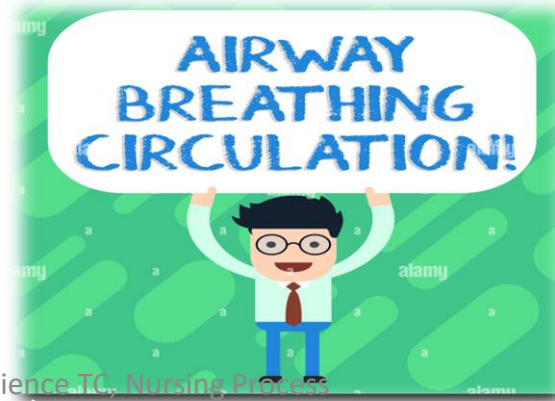
2. ABCS

- Airway, breathing, and circulation, otherwise known by the mnemonic “ABCs,” are another **foundational element** to assist the nurse in prioritization.
- Like Maslow’s hierarchy, using the ABCs to guide decision-making concentrates on the most critical needs for preserving human life.



2. ABCS...cont'D

- If a patient does not have a patent airway, is unable to breathe, or has inadequate circulation, very little of what else we do matters.
- The patient's **ABCs** are reflected in Maslow's foundational level of physiological needs and **direct critical nursing actions** and **timely interventions**.



3. CURE



3. CURE

❑ The CURE hierarchy uses the acronym “**CURE**” to guide prioritization based on identifying the differences among

1. Critical needs,
2. Urgent needs,
3. Routine needs, and
4. Extras.



1/4/2024

❑ **Critical** patient needs require immediate action.

❑ **Examples** of critical needs align with the ABCs and Maslow's physiological needs, such as symptoms of respiratory distress, chest pain, and airway compromise.

- No matter the complexity of their shift, nurses can be assured that addressing patients' **critical needs** is the correct prioritization of their time and energies.



3. *CURE...Cont'd*

- After critical patient care needs have been addressed, nurses can then address “**urgent**” needs.
- Urgent needs are characterized as needs that cause patient discomfort or place the patient at a significant safety risk.



3. *CURE...Cont'd*

- The third part of the CURE hierarchy reflects “routine” patient needs.
- Routine patient needs can also be characterized as “typical daily nursing care” because the majority of a standard nursing shift is spent addressing routine patient needs.



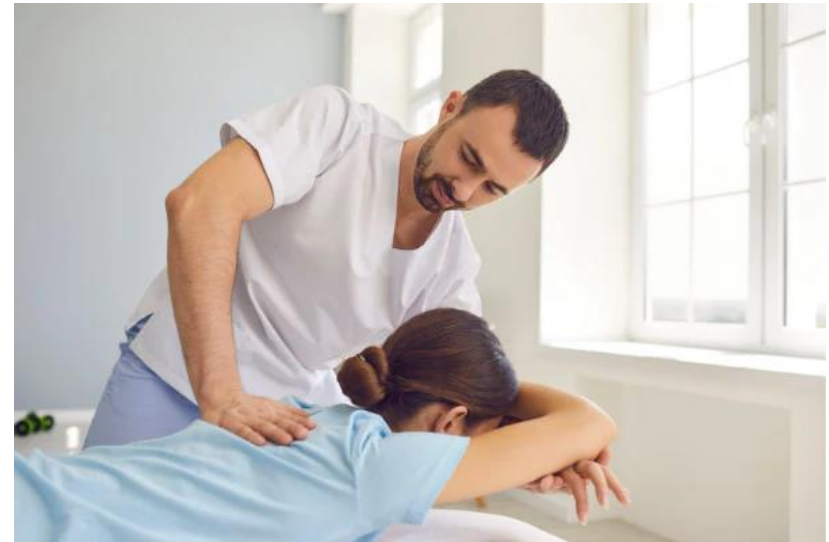
3. *CURE...Cont'd*

- **Examples of routine daily nursing care** include actions such as
 - ✓ Administering medication
 - ✓ Wound care
 - ✓ Positioning of client and
 - ✓ Performing physical assessment
- Although a nurse's typical shift in a hospital setting includes these routine patient needs, they do not supersede critical or urgent patient needs.



3. *CURE...Cont'd*

- **The final component of the CURE hierarchy is known as “extras.”**
- Extras refer to activities performed in the care setting to facilitate patient comfort but are not essential.



❑ Examples of extra activities include

- Providing a massage for comfort or
- Washing a patient's hair.
- If a nurse has sufficient time to perform extra activities, they contribute to a patient's feeling of satisfaction regarding their care, but these activities are not essential to achieve patient outcomes.



1/4/2024



Milki Health science TC, Nursing Process
By: Fantahun Guteta (BscN, Mph)



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4. Data CUES



4. Data Cues

- In addition to using the identified frameworks and tools to assist with priority setting, nurses must also look at their patients' **data cues** to help them identify care priorities.
- **Data cues** are pieces of significant clinical information that direct the nurse toward a potential clinical concern or a change in condition.





4. Data Cues...Cont'd

❑ **For example,**

- ✓ Have the patient's vital signs worsened over the last few hours?
- ✓ Is there a new laboratory result that is concerning?

- Data cues are pieces of information or observations that a nurse gathers about a patient's
 - ✓ Health status,
 - ✓ Behavior, or
 - ✓ Response to treatment.



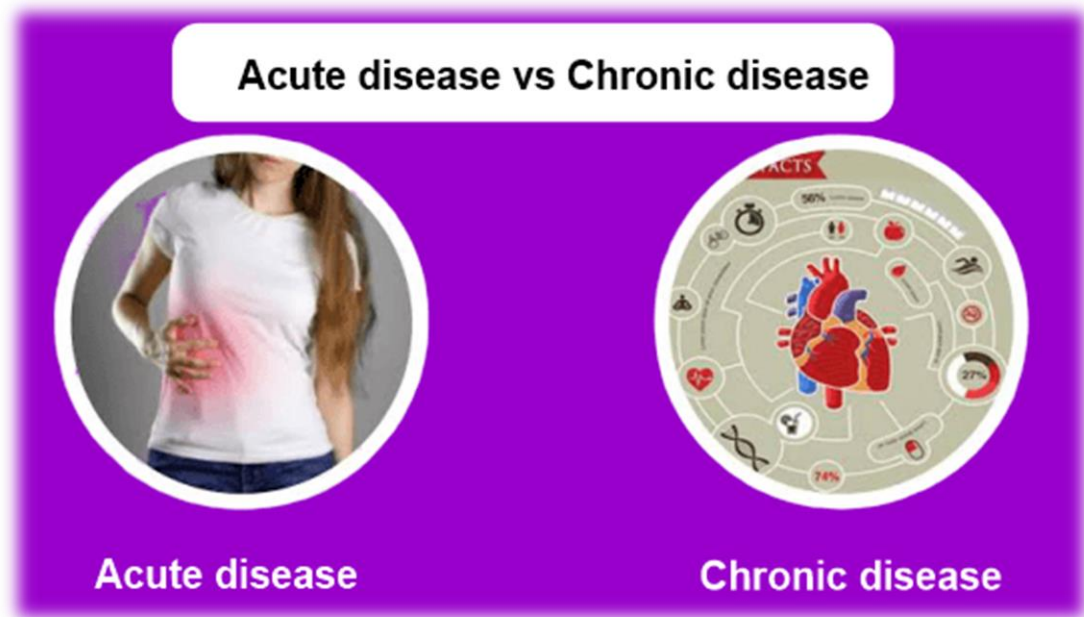
4. Data Cues...Cont'd

□ Common categories of data clues include:

- ✓ Acute versus chronic conditions,
- ✓ Actual versus potential problems,
- ✓ Unexpected versus expected conditions,
- ✓ Information obtained from the review of a patient's chart, and diagnostic information.

1. Acute Versus Chronic Conditions

- ❖ A common data cue that nurses use to prioritize care is considering if a condition or symptom is **Acute** or **Chronic**.





4. Data Cues...Cont'd

2. Actual Versus Potential Problems

- Nursing diagnoses and the nursing care plan have significant roles in directing prioritization when interpreting assessment data cues.

3. Unexpected Versus Expected Conditions

- It is also important to consider if a client's signs and symptoms are “expected” or “unexpected” based on their overall condition.





4. *Data Cues...Cont'd*

- A. Unexpected conditions** are findings that are not likely to occur in the normal progression of an illness, disease, or injury. Unexpected findings often ***require immediate action by the nurse.***

- B. Expected conditions** are findings that are likely to occur or are anticipated in the course of an ***illness, disease, or injury.***



4. Data Cues...Cont'd

4. Handoff Report/Chart Review

- Additional data cues that are helpful in guiding prioritization come from information obtained during a handoff nursing report and review of the patient chart.
- These data cues can be used to establish a patient's baseline status and prioritize new clinical concerns based on **abnormal assessment findings**.

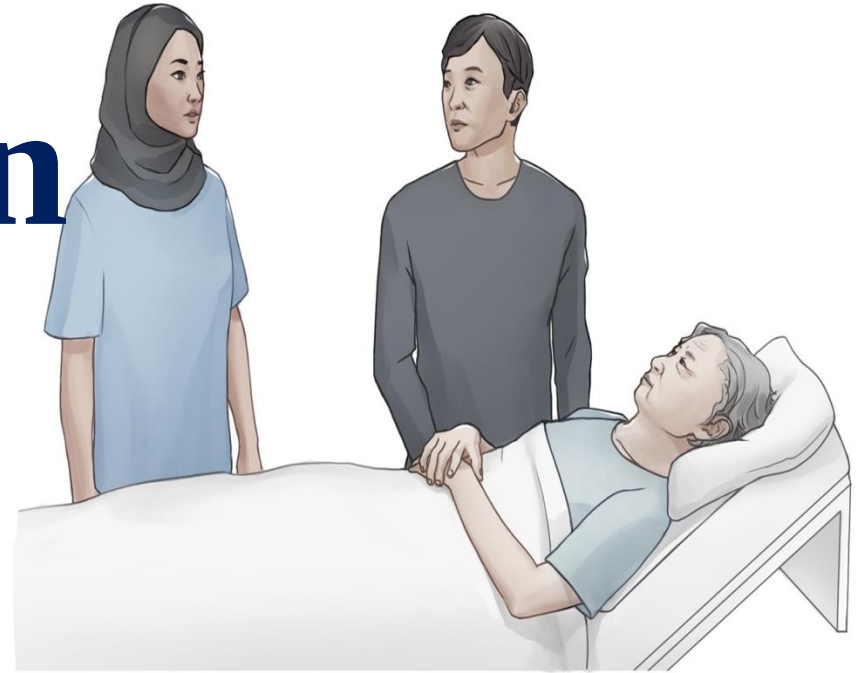


4. Data Cues...Cont'd

5. Diagnostic Information

- Diagnostic results are also important when prioritizing care.
- New abnormal laboratory results are typically flagged in a patient's chart.
- Newly reported abnormal results, such as **elevated blood levels** or **changes on a chest X-ray**, may indicate a patient's change in condition and require additional interventions.

5. Evaluation phase





5. Evaluation phase

- Evaluating is the **fifth step** of the nursing process.
- This final phase of the nursing process is vital to a positive patient outcome.
- **New problems** may be identified at this stage, and thus the process will start all over again.



Evaluation phase...cont'd

- The possible patient outcomes are generally explained under **three terms**:
 1. The patients condition **improved**,
 2. The patients condition **stabilized**, and
 3. The patients condition **worsened**.



Steps in Evaluation

- **Nursing evaluation includes:-**

- (1) Collecting data,
- (2) Comparing collected data with desired outcomes,
- (3) Analyzing clients response relating to nursing activities,
- (4) Identifying factors that contributed to the success or failure of the care plan,
- (5) Continuing, modifying, or terminating the nursing care plan, and
- (6) Planning for future nursing care.



Steps in Evaluation...Cont'd

1. Collecting Data

- The nurse recollects data so that conclusions can be drawn about whether goals have been fulfilled.
- It is usually vital to collect both objective and subjective data.
- Data must be documented concisely and accurately to facilitate the next part of the evaluating process.



Steps in Evaluation...Cont'd

2) Comparing Data with Desired Outcomes

- The documented goals and objectives of the nursing care plan become the standards or criteria by which to measure the client's progress whether the desired outcome has been met, partially met, or not met.
 - A. The goal was met**, when the client response is the same as the desired outcome.
 - B. The goal was partially met**, when either a short-term outcome was achieved but the long-term goal was not, or the desired goal was incompletely attained.
 - C. The goal was not met.**



Steps in Evaluation...Cont'd

3) Analyzing client's response relating to nursing activities

- It is also very important to determine whether the nursing activities had any relation to the outcomes whether it was *successfully accomplished or not.*



Steps in Evaluation...Cont'd

4) Identifying factors contributing to success or failure

- It is required to collect more data to confirm if the plan was successful or a failure.
- Different factors may contribute to the achievement of goals.
 - **For example,** the client's family may or may not be supportive, or the client may be uncooperative to perform such activities.



Steps in Evaluation...Cont'd

5) Continuing, Modifying, or Terminating the Nursing Care Plan

- ✓ The nursing process is dynamic and cyclical.
- ✓ If goals were not sufficed, the nursing process begins again from the first step.
- ✓ The plan of care may be adjusted based on new assessment data.
- ✓ Problems may arise or change accordingly.
- ✓ As clients complete their goals, new goals are set.
- ✓ If goals remain unmet, nurses must evaluate the reasons these goals are not being achieved and recommend revisions to the nursing care plan.

6) Planning for future nursing care.





Common challenges during the evaluation phase



Common challenges during the evaluation phase and how to overcome

❑ Challenge #1: Incomplete Documentation

❑ About the Challenge:

- Every nurse is responsible for documenting patient progress and other pertinent information.
- If one nurse fails to document and report patient **changes or progress**, or **to record laboratory or diagnostic test results**, it can lead to challenges when it is time for the evaluation phase.

Common challenges...Cont'd

❑ How to Overcome:

- ✓ The most effective way to overcome this challenge is to avoid it **happening altogether**.
- ✓ Be sure to document information about your patients during each shift.





Common challenges...Cont'd

❑ Challenge #2: Patient Frustration

❑ About the Challenge:

- ✓ In a perfect world, patients would be admitted to care, nursing interventions would be implemented, and we would see positive results.
- ✓ Unfortunately, it doesn't always work that way.
- ✓ One of the challenges nurses face in the evaluation phase is frustration related to slow progress or failing to meet goals.

❑ How to Overcome:

- Overcoming the challenges that occur when patients become frustrated takes patience and understanding.
- Take the time to talk to your patient.
- Assure them that progress does not always happen as quickly as we would like and encourage them to keep pressing forward.





Common challenges...Cont'd

❑ Challenge #3: Patients Withholding Information

❑ About the Challenge:

- It is not uncommon for patients to try and mask symptoms or deny concerns when nurses evaluate their progress.
- This is especially common when a patient has been hospitalized or in a care facility for an extended period and wants to return home.



Common challenges...Cont'd

□How to Overcome:

- In situations like this, a strong nurse-patient relationship and good communication skills are necessary.
- If you feel your patient is not being forthcoming about their progress, or perhaps new symptoms have emerged that they do not want to discuss, ask direct questions.
- Explain the importance of transparency when reporting progress, problems, or concerns.



Common challenges...Cont'd

□ Challenge #4: Family Denial of the Patient's Need for Continued Care

□ About the Challenge:

- ✓ As nurses, we naturally hope that interventions positively impact our patients and that we can see improvement when evaluating them.
- ✓ Family members hope to see their loved ones recover and return to normal, as well.
- ✓ A significant challenge nurses can face when evaluating patient progress occurs when the patient's status declines or there is little improvement and family members deny the reality of the patient's situation.

□How to Overcome:

- ✓ Many times, the fear of the unknown or lack of understanding is what causes family denial.
- ✓ If family members struggle with accepting the idea of continued care or changes in the plan of care, the nurse should acknowledge their concerns and offer support.
- ✓ When appropriate, talk with the patient and family together and discuss the previous plan of care and any suggestions for alterations or changes and the rationale for them

❑ Challenge #5: Patient wishing to terminate care before discharge goals are met

❑ About the Challenge:

- Nurses understand that patients respond differently to care with some progressing faster than others, and the evaluation process helps to identify those patients who need extra time or updated care plans.
- If patients slowly progress or fail to meet goals and expected outcomes, their frustration sometimes leads them to give up or desire to seek care elsewhere.



Common challenges...Cont'd

□ How to Overcome:

- ✓ When faced with this challenge, it is crucial for nurses to approach the patient with an attitude of empathy and attempt to discuss the patient's concerns.
- ✓ Depending on the patient's status and ability to understand, it may take some time and reinforced teaching to help them understand that slow progress is not failure.
- ✓ Explain your view of your patient's current status compared to his status on admission.



Common challenges...Cont'd

- Encourage the patient by assuring him that even slow progress is progress.
- It is necessary to document everything you discuss with the patient and his response to your instructions and education.
- If the patient decides to leave your facility's care, there is appropriate paperwork to be signed, called an A.M.A. (**Against Medical Advice**) discharge.
- The charge nurse or physician is usually responsible for having the patient sign this form and submitting it to administration.



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TAJAAJILA KENNINU

- ✓ Barnoota Dabalataa Kennu
- ✓ Exit Exam/ COC'f isin qopheessuu
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Thank you!



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